

Tonia Ojomo, RN, BSN, PMHNP

PSYCHIATRIC ASSESSMENT MASTERY



Featuring

DSM-5-TR Decision Trees

Includes Full MSE Checklists

Step-by-Step Interview Scripts

Clinical Reasoning & SOAP Guides

Download the Digital Toolkit & Templates at:

www.psychmastery.com

This workbook bridges the gap between textbook theory and real-world clinical practice. Built specifically for **PMHNP** students, **RNs**, and new practitioners, it provides practical scripts, templates, and decision-making frameworks to support safe, structured, and efficient psychiatric assessment.

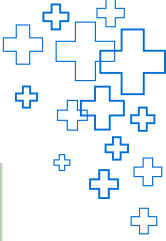
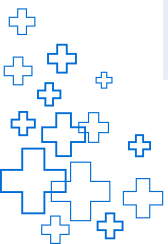
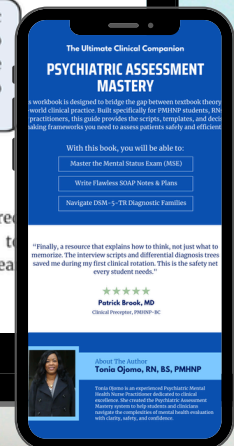
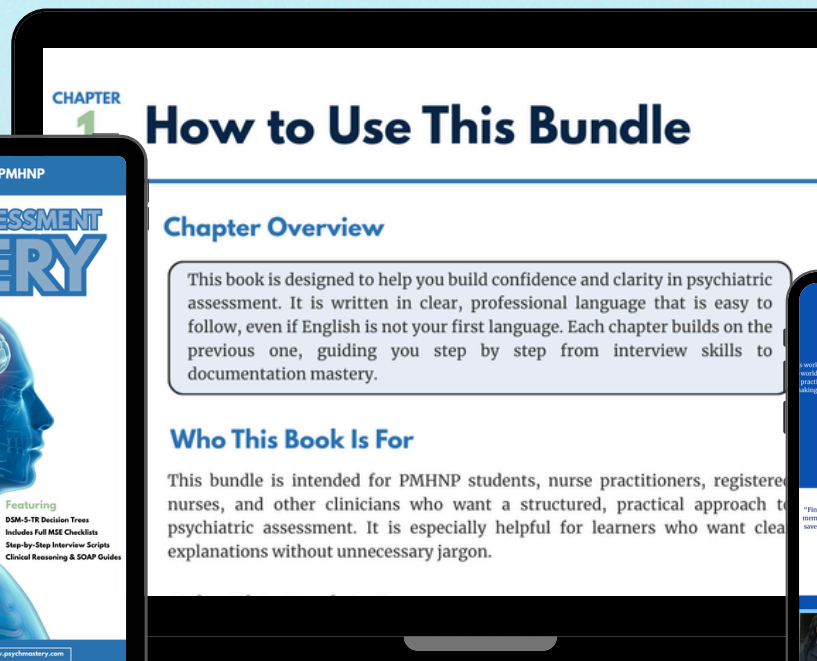
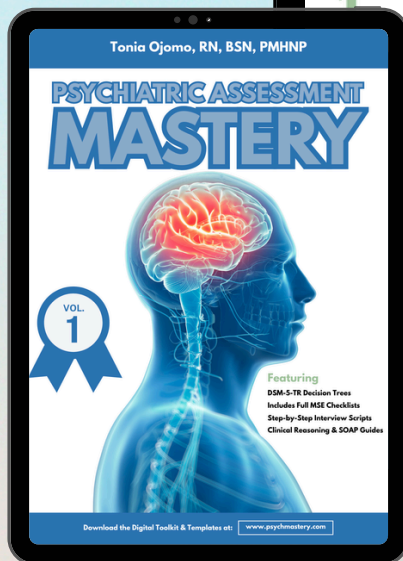


Table of Contents

Chapter 1 —	<u>How to Use This Bundle</u>	11
Chapter 2 —	<u>Psychiatric Assessment Basics</u>	13
Chapter 3 —	<u>Psychiatric History & HPI Mastery</u>	15
Chapter 4 —	<u>Mental Status Examination (MSE)</u>	20
Chapter 5 —	<u>Clinical Formulation & Case Conceptualization</u>	26
Chapter 6 —	<u>Risk Assessment & Safety Evaluation</u>	32
Chapter 7 —	<u>DSM-5-TR Diagnostic Groupings</u>	37
Chapter 8 —	<u>The Clinical Assessment (A)</u>	39
Chapter 9 —	<u>Treatment Planning (P)</u>	44
Chapter 10 —	<u>Documentation Mastery</u>	50
Chapter 11 —	<u>Clinical Interviewing</u>	55
Chapter 12 —	<u>Case Studies</u>	60
Chapter 13 —	<u>Skill Workbook</u>	69
Chapter 14 —	<u>Quick Reference Cheat Sheets</u>	77
Chapter 15 —	<u>Glossary</u>	80
Chapter 16 —	<u>Putting It All Together</u>	83
Chapter 17 —	<u>Final Review & Next Steps</u>	87
Chapter 18 —	<u>Professional, Ethical, and Legal Considerations</u>	90
Chapter 19 —	<u>FAQs, Common Mistakes, & Red Flags</u>	92
Chapter 20 —	<u>Capstone Toolkit</u>	96



SCAN THE QR CODE TO GET A DIGITAL BUNDLE



What This Is / What This Is Not



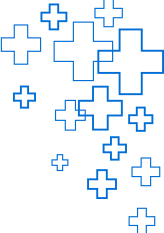
WHAT THIS IS

- ▶ **Volume 1 of Psychiatric Assessment Mastery:** A focused guide on Psychosis & Mood Disorders.
 - ▶ **A Clinical Workflow System:** Designed to improve psychiatric interviewing, MSE documentation, diagnostic reasoning, and **SOAP** writing.
 - ▶ **For Active Learners:** Built for PMHNP students, NPs, RNs, and clinicians who want a repeatable, structured framework for practice.
 - ▶ **Real-World Support:** A tool meant to be used during clinical practicum, exam preparation, and daily documentation.
-



WHAT THIS IS NOT

- ▶ **Not a Substitute for Supervision:** Always follow your institutional policies, clinical protocols, and attending physician/supervisor guidance.
 - ▶ **Not a DSM-5-TR Replacement:** This book teaches pattern recognition and clinical reasoning. You must still verify specific criteria in the full DSM-5-TR text.
 - ▶ **Not a Theory Book:** This is not a text you read once and put away. It is a workbook designed for active use and repetition.
-



How to Think Like a Psychiatric Clinician

A Simple Guide for Clear Clinical Thinking

Thinking like a psychiatric clinician does not mean using big medical words or memorizing the DSM. It means learning how to:

- ✓ Listen carefully
- ✓ Notice patterns
- ✓ Ask the right questions
- ✓ Stay calm and curious
- ✓ Connect symptoms to possible explanations

Start With Curiosity, Not Diagnosis

One of the most common mistakes new clinicians make is trying to name a diagnosis too quickly. Your job is to understand the patient's story first. The diagnosis comes later.

Instead of asking:

What diagnosis is this?

Ask:

What is this person experiencing right now?

Use This Simple 4-Step Thinking Process

Step 1: Listen to the Story

- ✓ Listen carefully
- ✓ Notice patterns
- ✓ Ask the right questions
- ✓ Stay calm and curious

Step 2: Observe What You See

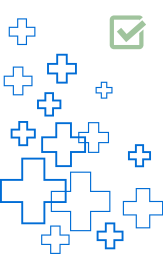
- ✓ Appearance and behavior
- ✓ Eye contact, speech speed and tone
- ✓ Mood and emotional expression
- ✓ Movement (restless, slowed, calm)

Step 3: Identify Key Patterns

- ✓ Do the symptoms cluster together?
- ✓ Is there a clear timeline?
- ✓ Is there an immediate safety risk?
- ✓ How is the patient's functioning being affected?

Step 4: Ask, "What Could Explain This?"

- ✓ Could this be depression?
- ✓ Medication side effects?
- ✓ Anxiety or Trauma-related?
- ✓ Medical causes or Substance use?



THINK IN POSSIBILITIES, NOT ABSOLUTES

Psychiatric assessment is not about being 100% certain at the first visit. It is about:

- ✓ Identifying the most likely possibilities
- ✓ Ruling out dangerous conditions
- ✓ Creating a safe plan
- ✓ Reassessing over time

SAFETY ALWAYS COMES FIRST

If safety is a concern, address it before anything else. No matter the diagnosis, always assess for:

- ✓ Suicidal thoughts
- ✓ Homicidal thoughts
- ✓ Psychosis
- ✓ Severe mood instability
- ✓ Substance withdrawal



Use Plain Language in Your Own Mind

This patient feels sad most days, has low energy, poor sleep, and has stopped enjoying life.



The above is better thinking than:

Patient meets criteria for major depressive disorder.

Clinical Thinking Improves with Practice

Confidence grows with repetition. No one becomes a strong psychiatric clinician overnight. You will improve by:

- ✓ Practicing assessments
- ✓ Reviewing cases
- ✓ Using templates and checklists
- ✓ Reflecting on what worked and what did not

Final Reminder



Clear thinking

Careful listening

Organized notes

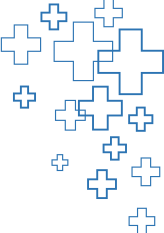


Perfect English.

Sound like a textbook

All the answers

That is what makes a good psychiatric clinician.



Quick Start: The 3-Step Assessment Workflow

1

Start with S (Subjective)

- ▶ **Chief Complaint:** Why are they here today?
- ▶ **HPI:** Onset, duration, severity, and triggers.
- ▶ **Safety:** Ask directly about SI/HI.
- ▶ **History:** Review medical, psychiatric, and social context.

2

Move to O + MSE (Objective)

- ▶ **Appearance & Behavior:** Grooming, eye contact, agitation.
- ▶ **Speech & Affect:** Rate, tone, volume, and emotional range.
- ▶ **Cognition:** Thought process, insight, and judgment.
- ▶ **Safety Signs:** Objective markers of risk or intoxication.

3

Write A + P (Assessment + Plan)

- ▶ **Diagnosis:** Primary diagnosis + key differentials/rule-outs.
- ▶ **Risk Level:** Low, Moderate, or High (with rationale).
- ▶ **Plan:** Medications, therapy, education, and follow-up timing.

How to Use This Workbook in Clinicals



BEFORE THE PATIENT ENCOUNTER

- ☒ **Review Question Banks:** Check the "Symptom Quick Prompts" (Chapter 13) for the patient's presenting problem (e.g., Depression vs. Anxiety).
 - ☒ **Refresh the MSE:** Skim the "Mental Status Exam" checklist (Chapter 4) to remember what to observe.
 - ☒ **Prep Your Template:** Open the HPI or SOAP template so you are ready to take structured notes.
-



AFTER THE PATIENT ENCOUNTER

- ☒ **Draft the Note:** Fill in the SOAP template while the details are fresh in your mind.
 - ☒ **Check Your Wording:** Use the "Phrase Bank" (Chapter 19) to convert vague notes into professional medical language.
 - ☒ **Verify Safety:** Double-check that you documented SI/HI assessment and Risk Level explicitly.
 - ☒ **Self-Reflect:** Identify one specific area (e.g., "asking about trauma") to improve in the next encounter.
-

How to Use This Bundle

Chapter Overview

This book is designed to help you build confidence and clarity in psychiatric assessment. It is written in clear, professional language that is easy to follow, even if English is not your first language. Each chapter builds on the previous one, guiding you step by step from interview skills to documentation mastery.

Who This Book Is For

This bundle is intended for PMHNP students, nurse practitioners, registered nurses, and other clinicians who want a structured, practical approach to psychiatric assessment. It is especially helpful for learners who want clear explanations without unnecessary jargon.

Who This Book Is For

Recommended approach:

- ▶ Read one chapter at a time rather than rushing through the book
- ▶ Practice using the worksheets and templates provided
- ▶ Repeat the same exercises with different fictional patients to build speed
- ▶ Use the cheat sheets before and after clinical encounters

How the Book Is Organized

The book moves from foundational concepts to advanced application:

- ▶ Chapters 1–4: Core foundations and assessment basics
- ▶ Chapters 5–8: Risk assessment, diagnosis, and treatment planning
- ▶ Chapters 9–12: Documentation mastery and skill-building workbooks
- ▶ Chapters 13–16: Quick references, full examples, and implementation guidance

A Note on Language and Learning Style

This book intentionally uses clear, straightforward language. The goal is understanding, not memorization. If a clinical term feels confusing, use the glossary and focus on describing what you actually observe in practice.

Chapter Summary

In this opening chapter, you learned how to use this bundle effectively and how the content is organized. Approach the material step by step, practice consistently, and use the templates as support tools. With repetition, the structure of psychiatric assessment will become second nature.

How to Use This Book Effectively

Chapter Overview

This chapter shows you how to use this book in a practical, step-by-step way. You will learn how to study efficiently, how to practice the skills in real settings, and how to use the templates and quick-reference tools to build confidence over time.

Who This Book Is For

- ▶ How the chapters are organized and why order matters
- ▶ How to learn psychiatric assessment through practice (not memorization)
- ▶ How to use templates, checklists, and examples to improve faster
- ▶ A simple weekly study plan you can follow
- ▶ How to apply this book during clinicals, work, and exam prep

1. Read in Order: This Is a Skill-Building System

This book is organized from basic concepts to more advanced clinical reasoning. Each chapter builds on the chapter before it. If you skip ahead too quickly, later sections may feel harder than they need to be.

Recommended reading order:

1. Foundations and the SOAP structure
2. Subjective assessment (what the patient tells you)
3. Objective assessment and the Mental Status Exam (what you observe)
4. Clinical assessment and diagnosis (what it means)
5. Treatment planning (what happens next)
6. Documentation, case studies, workbook, and cheat sheets

2. Learn by Doing: Practice Beats Memorizing

Psychiatric assessment is not a subject you master by reading once. It is a clinical skill. The fastest way to improve is to practice with structure. Use the examples as models, then write your own **HPIs**, **MSEs**, and **SOAP** notes using the templates.

Simple Practice Rule

For every concept you learn, complete one short written practice note. Even 10 minutes of practice builds confidence.

3. Use the Templates to Stay Organized

The templates in this book are designed to reduce stress and save time. They help you remember what to ask, what to observe, and how to document your findings clearly. When you are new, structure is your best support.

Best Practice

Start each assessment by opening the template and filling it in as you go. Over time, you will rely on it less because the structure becomes natural.

4. Keep the Cheat Sheets Nearby

Cheat sheets are meant for real-world use. Many learners keep them on their phone, print them for clinicals, or place them in a binder. Use them to refresh your memory quickly, especially for the Mental Status Exam, red flags, and documentation phrases.

5. A Simple Weekly Study Plan

If you want a simple routine, follow this weekly plan. Adjust the pace based on your schedule.

Day 1: Read one chapter section (20–30 minutes).

Day 2: Practice writing one HPI or one MSE (10–15 minutes).

Day 3: Review a cheat sheet and rewrite key phrases in your own words (10 minutes).

Day 4: Complete one mini-case or case study question (15–20 minutes).

Day 5: Review your notes and identify what you still find confusing (10 minutes).

6. Use This Book During Clinicals and Real Encounters

This book is designed for real use, not just reading. During clinicals or work (when appropriate), use the symptom question banks, MSE structure, and documentation templates to guide your assessment. If you are not allowed to use materials during patient encounters, use the book before and after the encounter to prepare and document.

Chapter Summary

This chapter gave you a practical method for using this book effectively. You learned to follow the chapter order, focus on practice rather than memorization, and rely on templates and cheat sheets to stay organized. With consistent repetition, psychiatric assessment becomes clearer, faster, and more confident.

Practice Exercises

1. Write a short goal for how you will use this book over the next 7 days.
2. Choose one template (HPI, MSE, or SOAP). Practice filling it in using a fictional patient.
3. Pick one cheat sheet and review it for 5 minutes. Then write three key points you remember.

The Structure of Psychiatric Evaluation (SOAP Method)

Chapter Overview

This chapter introduces the SOAP method, a simple and widely used structure for psychiatric evaluations. SOAP helps you stay organized, write clear notes, and communicate effectively with other clinicians. You will learn what belongs in each section and how the parts work together in real practice.

What You Will Learn

- ▶ What **SOAP** stands for and why it is used
- ▶ What information belongs in Subjective (**S**)
- ▶ What information belongs in Objective (**O**) and the Mental Status Exam (**MSE**)
- ▶ What belongs in Assessment (**A**): clinical meaning, diagnoses, and risk
- ▶ What belongs in Plan (**P**): safe next steps and follow-up
- ▶ How to write a simple **SOAP** note using a real-world structure

1. Why Structure Matters in Psychiatric Assessment

Psychiatric interviews can feel open-ended because people describe emotions, thoughts, and life experiences in many different ways. Without a structure, it is easy to feel overwhelmed or to miss key information. **SOAP** gives you a clear frame so you can gather the right details, interpret them clinically, and document them in a professional way.

2. Subjective (S): What the Patient Reports

The Subjective section includes information that comes directly from the patient (or caregiver). It is the patient's story, stated in their words as much as possible.

Include:

- ▶ What **SOAP** stands for and why it is used
- ▶ What information belongs in Subjective (S)
- ▶ What information belongs in Objective (O) and the Mental Status Exam (MSE)
- ▶ What belongs in Assessment (A): clinical meaning, diagnoses, and risk
- ▶ What belongs in Plan (P): safe next steps and follow-up
- ▶ How to write a simple **SOAP** note using a real-world structure
- ▶ Social history and stressors (as reported)
- ▶ Safety report (SI/HI) in the patient's words

3. Objective (O): What You Observe

The Objective section includes information you observe as the clinician. This is where you document the Mental Status Exam (MSE), behavior, and any observable findings. Objective data should be descriptive and non-judgmental.

Include:

- ▶ Appearance (grooming, hygiene, eye contact)
- ▶ Behavior (calm, restless, cooperative, guarded)
- ▶ Speech (rate, volume, tone)
- ▶ Mood (patient's words) and affect (your observation)
- ▶ Thought process and thought content
- ▶ Perception (hallucinations)
- ▶ Cognition (orientation, attention, memory)
- ▶ Insight and judgment

4. Assessment (A): What It Means Clinically

The Assessment section is your clinical interpretation. This is where you summarize the key findings, state your most likely diagnoses, list differential diagnoses, document rule-outs, and describe risk level. Assessment should connect symptoms to clinical conclusions.

Include:

- ▶ Brief symptom summary (most important points)
- ▶ Primary diagnosis (most likely diagnosis)
- ▶ Differential diagnoses (other reasonable possibilities)

- ▶ Rule-outs (conditions considered but not supported)
- ▶ Risk assessment (SI/HI, psychosis, substance use, protective factors)

5. Plan (P): What Happens Next

The Plan section outlines the next steps. A strong plan is safe, realistic, and patient-centered. It may include medications, therapy recommendations, labs, referrals, education, safety planning, and follow-up timing.

Include:

- ▶ Medication plan (if appropriate): name, dose, reason, education
- ▶ Therapy plan: type of therapy and frequency (as appropriate)
- ▶ Labs or medical work-up (if needed)
- ▶ Referrals (therapy, psychiatry, primary care, substance programs)
- ▶ Patient education (sleep, coping skills, lifestyle)
- ▶ Safety plan (if risk present)
- ▶ Follow-up timeframe and what to monitor

6. Mini SOAP Example (Simple Model)

S

Patient reports increased anxiety for 3 weeks with restlessness, muscle tension, and poor sleep. Denies panic attacks. Denies suicidal or homicidal thoughts.

O

Calm and cooperative. Speech normal rate and tone. Mood “anxious.” Affect constricted but appropriate. Thought process logical and goal-directed. No hallucinations noted. Alert and oriented ×4.

A

Symptoms consistent with Generalized Anxiety Disorder. No SI/HI. Risk low. Rule out medical contributors if symptoms persist.

P

Recommend CBT for anxiety, coping skills practice, reduce caffeine, and follow up in 3–4 weeks. Review safety plan and return precautions.

Chapter Summary

In this chapter, you learned how the **SOAP** method provides a clear structure for psychiatric assessment and documentation. You reviewed what belongs in each section and how the parts work together to create a complete clinical picture. **SOAP** helps you stay organized, communicate clearly, and support safe clinical decisions.

Practice Exercises

1. Using the mini **SOAP** example, rewrite the note in your own words while keeping the same structure.
2. Choose a symptom (insomnia, low mood, or panic). Write one short sentence for **S**, **O**, **A**, and **P**.
3. Identify which **SOAP** section each item belongs to: patient quote, affect description, diagnosis, follow-up plan.

The Subjective Assessment (S)

Chapter Overview

The Subjective section captures the patient's story in their own words. In this chapter, you will learn how to open the interview, gather a clear History of Present Illness (HPI), ask symptom-focused questions in simple language, and document the information in a professional, organized way.

What You Will Learn

- ▶ How to start the interview and build rapport
- ▶ How to document the chief complaint and HPI clearly
- ▶ Practical symptom question banks (clear English, ESL-friendly)
- ▶ Key history areas: psychiatric, medical, family, social, substance use, trauma
- ▶ Strengths and protective factors
- ▶ Simple documentation tips that improve note quality

1. Start with Rapport and Psychological Safety

Many patients feel nervous, ashamed, or unsure during a psychiatric assessment. Your first goal is to create safety. When patients feel safe, they share more accurate information. When they feel judged, they may minimize symptoms or withhold details.



Opening script (use word-for-word)

Thank you for being here today. I'd like to understand what you've been experiencing. There are no right or wrong answers. I'm here to listen, and you can go at your own pace.

If the patient seems guarded, you can add: "Some questions may feel personal. You can tell me what you feel comfortable sharing today."

2. Chief Complaint (CC)

The **Chief Complaint (CC)** is a short statement of why the patient came in. Whenever possible, document it in the patient's own words.

Examples:

- ▶ "I can't sleep."
- ▶ "My anxiety is out of control."
- ▶ "I feel depressed."
- ▶ "I'm hearing voices."

3. History of Present Illness (HPI)

The HPI explains how symptoms started, how they changed over time, and how they impact daily functioning. A strong HPI helps you form a clear differential diagnosis.

Include:

- ▶ When did symptoms start?
- ▶ How long have they lasted?
- ▶ How severe are they?
- ▶ What makes symptoms better or worse?
- ▶ How do symptoms affect work, school, relationships, and self-care?
- ▶ Has this happened before?
- ▶ Any safety concerns (SI/HI)?



HPI example (model wording)

The patient reports low mood, low energy, and poor sleep for 6 weeks with reduced interest in usual activities. Symptoms worsen in the morning and are affecting work performance. No history of mania. Denies SI/HI.

4. Symptom Question Banks (Clear English)

These questions are written in simple language. Use them as a guide, not a script. Choose the questions that match the patient's main concerns.

Depression

- ▶ How has your mood been recently?
- ▶ Have you lost interest in things you usually enjoy?
- ▶ How is your sleep?
- ▶ How is your energy?
- ▶ How is your appetite or weight?
- ▶ Do you feel hopeless or guilty?
- ▶ Have you had thoughts of harming yourself?

Anxiety

- ▶ Do you feel nervous or tense most days?
- ▶ Do you worry a lot, even when you try not to?
- ▶ Does the worry feel hard to control?
- ▶ Do you feel restless or on edge?
- ▶ Do you have physical symptoms like muscle tension or stomach upset?
- ▶ Do you avoid places or situations because of anxiety?

Panic

- ▶ Do you have sudden episodes of intense fear?
- ▶ During these episodes, do you feel short of breath, dizzy, or like your heart is racing?
- ▶ Do you worry about having another episode?
- ▶ Do you avoid places because you fear a panic attack?

PTSD / Trauma

- ▶ Have you experienced something very scary or upsetting?
- ▶ Do you have nightmares or intrusive memories?
- ▶ Do you feel on guard or easily startled?
- ▶ Do you avoid reminders of what happened?
- ▶ Do you feel emotionally numb or disconnected at times?

Mania / Hypomania

- ▶ Have you had times when you needed much less sleep but still felt energetic?
- ▶ Have you felt unusually confident or more active than normal?
- ▶ Have you been talking more than usual?
- ▶ Have you been more impulsive with money, sex, or decisions?

Psychosis

- ▶ Do you hear voices or see things other people do not?
- ▶ Do you feel like someone is watching you or trying to harm you?
- ▶ Do you have beliefs that others say are not true?
- ▶ Do you ever feel your thoughts are being controlled?

Substance Use

- ▶ Do you use alcohol, cannabis, stimulants, opioids, or other substances?

- ▶ How often do you use, and how much?
- ▶ When was your last use?
- ▶ Has substance use caused problems at work, home, or with your health?
- ▶ Have you tried to cut down and found it difficult?

5. Key History Areas

After the HPI, collect key history information to understand patterns, risks, and contributing factors. Keep questions respectful and non-judgmental.

5.1 Past Psychiatric History

Ask about prior diagnoses, hospitalizations, medications, therapy, suicide attempts, and self-harm. Also ask what treatments helped and what did not help.



Documentation example

History of depression and anxiety. No hospitalizations. Prior sertraline trial discontinued due to nausea. No past suicide attempts.

5.2 Medical History and Current Medications

Ask about chronic conditions, current medications, allergies, and major surgeries. Medical issues can contribute to psychiatric symptoms, and medication interactions matter.

5.3 Family History

Ask about mood disorders, anxiety, psychosis, substance use, psychiatric hospitalizations, and suicide in close relatives. Family patterns can raise or lower the likelihood of certain diagnoses.

5.4 Social History

Cover living situation, relationships, work or school, stressors, finances, legal concerns, and social supports. Social context often explains triggers and barriers to treatment.

5.5 Trauma History

Ask in a sensitive way and do not force details. You can say: “Some people have been through difficult experiences that still affect them. Has anything like that happened to you?” If the patient shares trauma, focus on current symptoms and safety.

6. Strengths and Protective Factors

A complete assessment includes strengths, coping skills, and protective factors. These improve engagement and support risk evaluation.

Helpful Questions:

- ▶ What helps you cope when you are stressed?
- ▶ Who supports you?
- ▶ What are you most proud of about yourself?
- ▶ What keeps you going when things feel hard?

7. Documentation Tips (Simple and Professional)

Good documentation is clear, specific, and organized. Use the patient’s words when appropriate and avoid judgmental language.

Practical Tips:

- ▶ Use short sentences and plain words.
- ▶ Focus on timeframes (when it started, how long it lasted).
- ▶ Describe impact on function (work, school, relationships).
- ▶ Document safety clearly (SI/HI).
- ▶ Keep quotes brief and relevant.

Chapter Summary

In this chapter, you learned how to complete the Subjective portion of a psychiatric assessment. You practiced how to open the interview, document the chief complaint and HPI, and use symptom question banks in clear English. You also reviewed key history areas and the importance of strengths and protective factors.

Practice Exercises

1. Write a short HPI using the checklist in this chapter for a fictional patient with insomnia and anxiety.
2. Choose one symptom area (depression, anxiety, PTSD, mania, psychosis). Write five questions you would ask.
3. Document two protective factors for a fictional patient and explain why they matter for safety.

The Objective Assessment (O) & Mental Status Exam (MSE)

Chapter Overview

The Objective section includes what you observe as the clinician. The core of the Objective section is the Mental Status Exam (MSE). In this chapter, you will learn the parts of the MSE, how to describe findings in a clear and professional way, and how to avoid judgmental language. You will also see model wording you can use in your own notes.

What You Will Learn

- ▶ What belongs in the Objective (O) section
- ▶ The purpose of the Mental Status Exam (MSE)
- ▶ How to document MSE findings clearly and objectively
- ▶ Common MSE terms and what they mean (in plain English)
- ▶ Model MSE examples you can copy and adapt

1. What Is the Objective (O) Section?

The Objective section includes information you can observe or measure. In psychiatric settings, this usually means observations from the interview and the Mental Status Exam. Objective documentation should focus on what you see and hear, not assumptions about why it is happening.

Objective Writing Rule

Describe behavior and speech. Avoid labels like “manipulative,” “attention-seeking,” or “lazy.” Instead, document what the patient did or said

2. What Is the Mental Status Exam (MSE)?

The Mental Status Exam is a structured way to describe the patient's current functioning. It is a "snapshot" of how the patient presents during the assessment. The MSE supports diagnosis, risk evaluation, and communication with other clinicians.

3. MSE Components (Step-by-Step)

A complete MSE typically includes the areas below. You do not need to write long paragraphs for each section. Clear, short phrases are best.

3.1 Appearance

Describe grooming, hygiene, clothing, posture, and eye contact. Keep it factual.

Examples:

- ▶ Clean and well-groomed; appropriate clothing
- ▶ Disheveled; poor hygiene noted
- ▶ Limited eye contact; posture slumped

3.2 Behavior / Psychomotor Activity

Describe level of activity, cooperation, agitation, or slowing.

Examples:

- ▶ Calm and cooperative
- ▶ Restless, fidgeting in chair
- ▶ Psychomotor slowing observed; movements slowed
- ▶ Agitated; pacing in room

3.3 Speech

Document rate, volume, tone, and fluency.

Examples:

- ▶ Normal rate and volume
- ▶ Rapid, pressured speech
- ▶ Soft, slowed speech
- ▶ Speech loud; difficult to redirect

3.4 Mood

Mood is the patient's reported emotional state. Use the patient's words when possible.

Examples:

- ▶ Mood: "anxious"
- ▶ Mood: "depressed"
- ▶ Mood: "fine"
- ▶ Mood: "irritable"

3.5 Affect

Affect is what you observe: the emotional expression on the patient's face and in their voice. Comment on range and appropriateness.

Examples:

- ▶ Affect constricted but appropriate
- ▶ Affect flat
- ▶ Affect full range
- ▶ Affect labile

3.6 Thought Process

Thought process is how thoughts are organized and connected. Focus on flow and logic.

Examples:

- ▶ Logical and goal-directed
- ▶ Tangential
- ▶ Circumstantial
- ▶ Disorganized

3.7 Thought Content

Thought content is what the patient is thinking about. Assess for suicidal ideation (SI), homicidal ideation (HI), delusions, obsessions, and paranoia.

Examples:

- ▶ Denies SI/HI

- ▶ Reports passive suicidal thoughts without plan
- ▶ Paranoid thoughts reported
- ▶ Delusional beliefs noted

3.8 Perception

Assess for hallucinations (auditory, visual) or perceptual disturbances.

Examples:

- ▶ Denies hallucinations
- ▶ Reports hearing voices
- ▶ No response to internal stimuli observed

3.9 Cognition

Briefly document orientation, attention, memory, and concentration when relevant.

Examples:

- ▶ Alert and oriented ×4
- ▶ Attention mildly impaired; distracted
- ▶ Memory grossly intact

3.10 Insight

Insight reflects whether the patient understands their symptoms and needs help.

Examples:

- ▶ Insight good
- ▶ Insight fair
- ▶ Insight poor; does not recognize symptoms

3.10 Insight

Insight reflects whether the patient understands their symptoms and needs help.

Examples:

- ▶ Insight good
- ▶ Insight fair

- Insight poor; does not recognize symptoms

4. Model MSE Examples (Copy and Adapt)

Example A (Stable Presentation)

Appearance clean and well-groomed. Calm and cooperative behavior. Speech normal rate and volume. Mood “okay.” Affect appropriate and congruent. Thought process logical and goal-directed. Thought content denies SI/HI; no delusions. Perception denies hallucinations. Cognition alert and oriented ×4. Insight fair. Judgment intact.

Example B (Anxiety Presentation)

Appearance appropriate; mildly tense posture. Behavior cooperative; restless with frequent fidgeting. Speech normal volume; slightly rapid. Mood “anxious.” Affect constricted but appropriate. Thought process logical; ruminative. Thought content denies SI/HI. Perception denies hallucinations. Cognition alert and oriented ×4. Insight good. Judgment intact.

Example C (Psychosis Presentation)

Appearance disheveled; hygiene poor. Behavior guarded; intermittently scanning room. Speech normal volume; delayed responses. Mood “fine.” Affect blunted. Thought process tangential at times. Thought content paranoid ideation reported. Perception reports auditory hallucinations. Cognition alert and oriented ×3. Insight poor. Judgment impaired.

5. Documentation Tips (Objective, Safe Language)

Use clear, observable descriptions. If you are unsure, document what you observed rather than guessing. When documenting safety concerns, be direct and specific.

Helpful documentation phrases:

- ▶ “Patient denies suicidal ideation, plan, or intent.”
- ▶ “Thought process logical and goal-directed.”
- ▶ “No hallucinations reported; no response to internal stimuli observed.”
- ▶ “Affect constricted but appropriate.”
- ▶ “Insight fair; judgment intact.”

Chapter Summary

In this chapter, you learned what belongs in the Objective section and how to complete a Mental Status Exam (MSE). You reviewed each MSE component in clear language and practiced model wording you can adapt for your own notes. A strong MSE is concise, objective, and clinically useful.

Practice Exercises

1. Write a brief MSE for a fictional patient who is depressed and withdrawn (8–10 lines).
2. Rewrite a judgmental phrase (example: “noncompliant”) into objective language.
3. Choose three MSE components and list two example descriptors for each.

Risk Assessment & Safety Evaluation

Chapter Overview

Risk assessment is a core responsibility in psychiatric care. In this chapter, you will learn how to assess safety in a structured way, including suicidal thoughts (SI), homicidal thoughts (HI), psychosis-related risk, substance-related risk, and protective factors. You will also learn how to document risk clearly and how to create a basic safety plan.

What You Will Learn

- ▶ How to ask about suicidal and homicidal thoughts in a clear, respectful way
- ▶ Key risk areas: ideation, plan, intent, means, history, and supports
- ▶ Protective factors and why they matter
- ▶ Warning signs that require urgent action
- ▶ How to document risk assessment in professional language
- ▶ Basic safety planning steps and follow-up guidance

1. Why Risk Assessment Matters

Risk assessment protects the patient and others. It also helps you make safe clinical decisions. A risk assessment should be completed whenever a patient presents with depression, severe anxiety, psychosis, intoxication, major stressors, recent losses, or any statement suggesting self-harm or harm to others.

Key Point

Asking about suicidal thoughts does not cause suicidal thoughts. Clear, respectful questions improve safety.

2. How to Ask About Safety (Simple Scripts)

Ask directly, calmly, and without judgment. Use a tone that communicates safety and support.

Suicide (SI) Questions:

- ▶ “Have you had thoughts of harming yourself?”
- ▶ “Have you wished you could go to sleep and not wake up?”
- ▶ “Have you thought about ending your life?”
- ▶ “Do you have a plan?”
- ▶ “Do you have intent to act on that plan?”

Homicide (HI) questions:

- ▶ “Have you had thoughts of harming someone else?”
- ▶ “Is there a specific person you want to harm?”
- ▶ “Do you have a plan or access to weapons?”
- ▶ “Do you feel you might act on these thoughts?”

3. Core Risk Domains (What You Must Assess)

Use a structured approach so you do not miss critical information. When SI or HI is present, assess the areas below.

3.1 Ideation

Determine if thoughts are present, how often they occur, and whether they are passive or active.

3.2 Plan

Ask whether the patient has a specific plan and how detailed it is.

3.3 Intent

Assess whether the patient intends to act on the plan. Intent is a key driver of risk.

3.4 Means and Access

Ask about access to weapons, medications, or other means. Access increases risk.

3.5 History

Ask about prior suicide attempts, self-harm, violence, hospitalizations, and recent crises. Past behavior increases future risk.

3.6 Substance Use

Substance use can increase impulsivity and reduce judgment. Assess intoxication, withdrawal, and recent use patterns.

4. Protective Factors (What Reduces Risk)

Protective factors do not eliminate risk, but they can reduce it. They also guide safety planning and treatment engagement.

Examples of protective factors:

- ▶ Supportive family or friends
- ▶ Strong reasons for living (children, faith, goals)
- ▶ Willingness to seek help
- ▶ Stable housing and employment
- ▶ Future orientation (plans, hope)
- ▶ Engagement in treatment and medication adherence

5. Warning Signs That Require Urgent Action

Some situations require immediate escalation to a higher level of care. When in doubt, prioritize safety and consult a supervisor or follow your clinical protocol.

High-concern indicators:

- ▶ Active suicidal or homicidal intent
- ▶ A specific plan with access to means
- ▶ Command hallucinations telling the patient to harm self or others
- ▶ Severe intoxication or withdrawal with impaired judgment
- ▶ Recent suicide attempt or violent act
- ▶ Inability to commit to safety and no reliable supports

6. How to Document Risk Assessment (Clear, Professional Language)

Document safety clearly and directly. If SI/HI is denied, document that it was assessed. If present, document the key domains: ideation, plan, intent, means, history, and protective factors.

Example: SI denied

Patient denies suicidal ideation, plan, or intent. Denies homicidal ideation. No hallucinations reported. Risk assessed as low at this time.

Example: Passive SI

Patient reports passive suicidal thoughts without plan or intent. Denies past attempts. Reports strong family support and future goals. Protective factors reviewed. Safety plan discussed. Risk assessed as low to moderate; close follow-up arranged.

Example: Active SI with plan

Patient reports active suicidal ideation with a plan and access to means. Endorses intent. Limited protective factors. Immediate escalation initiated per protocol.

7. Basic Safety Planning Steps

Safety planning is a brief, practical intervention that helps patients reduce risk and identify supports. Safety plans should be individualized and documented clearly.

Basic safety plan elements:

- ▶ Warning signs (what tells the patient they are getting worse)
- ▶ Internal coping strategies (what they can do alone)
- ▶ Social supports (who they can contact)

- ▶ Professional supports (clinic, therapist, crisis resources)
- ▶ Environment safety (reduce access to means)
- ▶ Follow-up plan (when and how care will continue)

8. Follow-Up Planning Based on Risk

Follow-up frequency should match risk level and symptom severity. When risk increases, follow-up should be sooner and more structured.

General guidance (adjust to policy and clinical judgment):

- ▶ Low risk: routine follow-up and clear return precautions
- ▶ Moderate risk: closer follow-up, safety plan, involve supports as appropriate
- ▶ High risk: urgent escalation to a higher level of care per protocol

Chapter Summary

In this chapter, you learned how to complete a structured risk assessment, including SI, HI, psychosis-related risk, substance-related risk, and protective factors. You reviewed warning signs that require urgent action and learned how to document risk assessment clearly. Safety planning is a practical tool that supports patient protection and ongoing care.

Practice Exercises

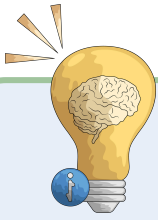
1. Write three different ways to ask about suicidal thoughts using a calm, respectful tone.
2. For a fictional patient with passive SI, document ideation, plan, intent, means, and protective factors in 4–6 sentences.
3. Create a simple safety plan outline (warning signs, coping strategies, supports, professional contacts, environment safety).

DSM-5-TR Diagnostic Groupings

Chapter Overview

This chapter teaches how clinicians identify the most appropriate DSM-5-TR diagnostic family using symptom patterns, onset, course, and context. You will learn to narrow diagnostic focus before consulting DSM criteria.

Key Diagnostic Families (High-Yield)	
Neuro-developmental Disorders	Early onset, lifelong patterns affecting attention, learning, social communication.
Schizophrenia Spectrum Disorders	Primary psychotic symptoms with impaired reality testing and functional decline.
Bipolar & Related Disorders	Episodes of mania or hypomania with mood elevation and decreased need for sleep.
Depressive Disorders	Persistent low mood or anhedonia without manic history.
Anxiety Disorders	Excessive fear or worry with avoidance and physical tension.
Trauma- & Stressor-Related Disorders	Symptoms following trauma or major stressor, including re-experiencing and hyperarousal.
Substance-Related Disorders	Symptoms directly related to intoxication or withdrawal.
Personality Disorders	Enduring interpersonal and emotional patterns beginning in early adulthood.



Clinical Tip

Always consider medical and substance-related causes before finalizing a primary psychiatric diagnosis.

Chapter Summary

Recognizing DSM-5-TR diagnostic groupings helps clinicians quickly narrow possibilities and apply criteria accurately. This pattern-recognition approach strengthens diagnostic reasoning and documentation.

The Clinical Assessment (A)

Chapter Overview

The Assessment section is where you explain what the information means clinically. In this chapter, you will learn how to summarize key findings, build a differential diagnosis, choose the most likely diagnosis, document rule-outs, and describe risk level. This chapter emphasizes clear clinical reasoning written in simple, professional language.

What You Will Learn

- ▶ What belongs in the Assessment (A) section
- ▶ How to write a concise clinical summary
- ▶ How to build a differential diagnosis (top 3 possibilities)
- ▶ How to document rule-outs (conditions considered but not supported)
- ▶ How to connect symptoms to diagnoses in clear reasoning
- ▶ Model Assessment examples you can copy and adapt

1. What Goes in the Assessment (A) Section?

The Assessment section is not a repeat of the Subjective and Objective sections. It is your interpretation. In a few short paragraphs or bullet points, you explain what you think is happening and why.

Include:

- ▶ Brief symptom summary (most important facts)
- ▶ Primary diagnosis (most likely diagnosis)
- ▶ Differential diagnoses (other reasonable possibilities)
- ▶ Rule-outs (diagnoses considered but not supported)
- ▶ Risk level and key safety findings

2. Write a Clear Clinical Summary

Start the Assessment with a short summary of the main symptoms, timeframe, and functional impact. Focus on what matters most for diagnosis and safety.



Clinical summary template

Patient reports [main symptoms] for [timeframe] with impact on [functioning].
MSE shows [key findings]. Safety: [SI/HI findings].

3. Differential Diagnosis Made Practical

A differential diagnosis is a list of possible diagnoses that could explain the patient's symptoms. You do not need a long list. In most clinical notes, list the top 2–3 most likely possibilities and explain briefly why.

Best practice

List the most likely diagnosis first, then 1–2 alternatives. Use simple reasoning: symptoms, duration, and key features that support or do not support each diagnosis.

4. Rule-Outs (How to Document Them)

A rule-out is a diagnosis you considered but the evidence does not support it right now. Rule-outs show critical thinking and help guide future assessment.



Rule-out phrasing examples

Rule out Bipolar II — no history of elevated mood or decreased need for sleep.
Rule out Substance-Induced Mood Disorder — symptoms not clearly linked to substance timing.
Rule out medical contributors — consider thyroid or anemia if symptoms persist.

5. Connect Symptoms to Diagnosis (Clear Reasoning)

When you state a diagnosis, include a brief explanation. This is especially important in school settings, clinical training, and case reviews. Your reasoning should connect: symptoms → duration → impact → key features.



Reasoning template

Diagnosis is supported by [symptoms] present for [timeframe] with [functional impact]. No evidence of [key exclusion], therefore [rule-out].

6. Integrate Risk into the Assessment

The Assessment should include risk level and the key findings that support it. Even when the patient denies SI/HI, document that risk was assessed. If risk is present, include the most important details (ideation, plan, intent, means, and protective factors).

7. Model Assessment Examples (Copy and Adapt)

Example A (Depression)

Patient reports low mood, low energy, poor sleep, and reduced interest in usual activities for 6 weeks with impact on work functioning. MSE shows constricted affect and slowed speech.

Denies SI/HI. Presentation most consistent with Major Depressive Disorder, moderate. Differential includes Generalized Anxiety Disorder (worry present but secondary). Rule out Bipolar II — no history of decreased need for sleep or elevated mood. Risk assessed as low at this time.

Example B (Anxiety)

Patient reports excessive worry most days for 3 months with restlessness, muscle tension, and sleep disruption. MSE shows anxious mood and constricted affect; thought process logical and goal-directed. Denies SI/HI. Symptoms most consistent with Generalized Anxiety Disorder. Differential includes Panic Disorder (no discrete panic episodes reported). Rule out medical contributors if symptoms persist. Risk assessed as low.

Example C (Psychosis)

Patient reports paranoia and auditory hallucinations with impaired functioning. MSE shows blunted affect, tangential thought process, and poor insight. Safety assessed; denies SI/HI, but judgment appears impaired and symptoms raise concern for risk if untreated. Presentation suggests a psychotic disorder. Differential includes substance-induced psychosis (substance history to clarify). Risk assessed as elevated due to psychosis and impaired judgment; follow-up and safety planning required.

8. Documentation Tips (High-Quality Assessment Writing)

Keep the Assessment concise. Avoid long repetition of the **HPI** or the full **MSE**. Focus on conclusions and your reasoning. Use neutral language and document uncertainty when needed (for example: “consider,” “rule out,” “suspect”).

Helpful phrases:

- ▶ “Symptoms most consistent with ...”
- ▶ “Differential includes ...”
- ▶ “Rule out ... due to ...”
- ▶ “Risk assessed as low/moderate/elevated based on ...”

Chapter Summary

In this chapter, you learned how to write the Assessment (A) section with clear clinical reasoning. You practiced summarizing key findings, listing differential diagnoses, documenting rule-outs, and integrating risk level. A strong Assessment is brief, logical, and clearly connected to the information gathered in the Subjective and Objective sections.

Practice Exercises

1. Write a clinical summary using the template for a fictional patient with panic symptoms for 4 weeks.
2. List a top 3 differential diagnosis for insomnia and explain one reason for each choice.
3. Write two rule-out statements using the model phrasing in this chapter.

Treatment Planning (P): Building a Patient- Centered Plan

Chapter Overview

The Plan section describes what happens next. A strong plan is safe, realistic, and centered on the patient's needs and preferences. In this chapter, you will learn how to build a complete treatment plan, including medications (when appropriate), therapy options, referrals, labs, education, safety planning, and follow-up timing.

What You Will Learn

- ▶ What belongs in the Plan (P) section
- ▶ How to write a plan that is safe, realistic, and patient-centered
- ▶ Core plan components: medications, therapy, referrals, labs, education, safety
- ▶ How to document follow-up and what to monitor
- ▶ Model treatment plan examples you can copy and adapt

1. The Purpose of the Plan (P) Section

The Plan section converts your assessment into action. It answers: What are we going to do now? Plans should be written clearly so the patient and other clinicians can understand the next steps. When possible, the plan should reflect shared decision-making.

2. Patient-Centered Planning (Shared Decision-Making)

A patient-centered plan considers the patient's goals, preferences, culture, access to care, and safety needs. Even when you recommend a specific treatment, invite the patient into the decision.

3. Core Components of a Complete Treatment Plan

Most psychiatric plans include several parts. You may not need every part for every patient, but you should consider them each time.

Helpful phrases:

- ▶ Medication plan (if appropriate)
- ▶ Therapy plan
- ▶ Labs or medical work-up (if indicated)
- ▶ Referrals (specialty care, community resources)
- ▶ Patient education (symptoms, coping, lifestyle)
- ▶ Safety planning (if risk present)
- ▶ Follow-up timeline and monitoring plan

4. Medication Planning (Clear, Safe Documentation)

If medications are part of the plan, document the medication name, dose, reason, and key education points. Also document what to monitor and when to follow up. Use clear language.



Medication Plan Template

Start/continue [medication] [dose] for [target symptoms]. Review risks/benefits and side effects. Patient education provided. Monitor [what to monitor]. Follow up in [timeframe].

5. Therapy Planning

Therapy is a core treatment for many conditions. If therapy is recommended, document the type of therapy and the focus. When therapy is already in place, document whether the patient is engaged and what goals are being addressed.

Examples:

- ▶ “Recommend CBT to address worry and avoidance.”
- ▶ “Recommend trauma-focused therapy to address intrusive memories and hypervigilance.”
- ▶ “Encourage ongoing therapy engagement; reinforce coping skills practice.”

6. Referrals and Care Coordination

Some patients need additional services such as therapy, psychiatry, primary care, substance treatment, or social supports. Document referrals clearly and include the reason for each referral.

Referral phrasing:

- ▶ “Refer to therapy for CBT to address anxiety symptoms.”
- ▶ “Coordinate with primary care to evaluate medical contributors to fatigue.”
- ▶ “Refer to substance use treatment program due to ongoing use and functional impairment.”

7. Labs and Medical Work-Up (When Indicated)

When symptoms could be related to a medical condition or medication effect, consider recommending appropriate medical evaluation or labs. Document the reason in clear terms (for example: fatigue, weight change, new mood symptoms, or cognitive concerns).

8. Patient Education (Make It Practical)

Education should be simple and realistic. Patients benefit from clear guidance on symptoms, coping strategies, sleep, routine, and return precautions. Document what you taught and what the patient understood.

Examples of education points:

- ▶ Sleep hygiene basics (consistent schedule, reduce caffeine, limit screens)
- ▶ Coping strategies (breathing, grounding, journaling, activity scheduling)
- ▶ Medication education (how to take it, what side effects to watch for)
- ▶ When to seek urgent help (worsening SI/HI, psychosis, severe side effects)

9. Safety Planning (When Risk Is Present)

If risk is present, document a safety plan and include clear follow-up and escalation guidance. Even with low risk, document return precautions so the patient knows what to do if symptoms worsen.

**Return Precautions Example**

Advised patient to seek urgent care or emergency services if suicidal thoughts worsen, if a plan develops, or if safety cannot be maintained.

10. Follow-Up and Monitoring

Follow-up timing depends on symptom severity, risk, and whether medications are started or changed. Document the timeline and what you plan to reassess at the next visit (symptoms, side effects, safety, functioning).

**Follow-up Phrasing**

Follow up in 2 weeks to reassess symptoms, sleep, and side effects. Follow up in 4 weeks to evaluate response and adjust plan as needed.

11. Model Plan Examples (Copy and Adapt)

Example A (Depression & Anxiety)

Start medication as appropriate for mood and anxiety symptoms. Recommend CBT to address negative thinking and worry. Provide education on sleep routine, activity scheduling, and coping skills practice. Review safety plan and return precautions. Follow up in 4 weeks to evaluate symptom response and side effects.

Example B (Anxiety, no medication change)

Recommend CBT for anxiety and avoidance. Encourage daily relaxation practice and reduce caffeine. Provide education on grounding skills and sleep hygiene. Follow up in 3–4 weeks to reassess symptom severity and functioning.

Example C (Elevated risk)

Risk elevated; safety plan reviewed and escalation steps discussed. Involve supports as appropriate and follow protocol for higher level of care. Arrange close follow-up and document return precautions clearly.

12. Documentation Tips (Make Plans Easy to Follow)

Write your plan as a set of clear actions. Avoid vague statements like “continue treatment” without details. Include what, why, and when: what you will do, why you are doing it, and when you will reassess.

Quick checklist:

- ▶ Medication: name, dose, reason, education, monitoring
- ▶ Therapy: type and focus
- ▶ Referrals: who and why
- ▶ Safety: plan and precautions
- ▶ Follow-up: timeframe and goals

Chapter Summary

In this chapter, you learned how to write the Plan (P) section as a safe, patient-centered set of next steps. You reviewed medication and therapy planning, referrals, education, safety planning, and follow-up. A strong plan is clear, realistic, and aligned with both clinical needs and patient goals.

Practice Exercises

1. Write a treatment plan for a fictional patient with insomnia and anxiety, including education and follow-up.
2. Create a plan checklist you would use for every new evaluation (5–8 bullet points).
3. Rewrite a vague plan statement (example: “continue meds”) into a clear action plan with monitoring and follow-up.

Documentation Mastery: SOAP Notes & Evaluation Writing

Chapter Overview

Good documentation is clear, organized, and clinically meaningful. In this chapter, you will learn how to write strong psychiatric notes using the SOAP structure, how to document evaluations in a professional tone, and how to avoid common documentation mistakes. You will also receive model phrasing you can copy and adapt for real-world practice.

What You Will Learn

- ▶ Why documentation matters in psychiatric care
- ▶ The difference between a **SOAP** note and a full psychiatric evaluation note
- ▶ How to write each **SOAP** section clearly (**S**, **O/MSE**, **A**, **P**)
- ▶ Safe, objective language (what to avoid and what to write instead)
- ▶ Model phrases and mini-templates you can copy and adapt

1. Why Documentation Matters

Documentation is communication. It helps other clinicians understand what happened, what you found, what you decided, and why. It also supports continuity of care, clinical safety, and professional accountability.

2. SOAP Notes vs. Full Psychiatric Evaluations

A **SOAP** note is commonly used for follow-up visits and many outpatient encounters. A full psychiatric evaluation is usually longer and often includes a more detailed history, diagnostic reasoning, and baseline **MSE**.

In Simple Terms

SOAP note: a structured clinical update (often shorter). Full evaluation: a comprehensive baseline assessment (often longer).

3. Writing Strong SOAP Notes (Step-by-Step)

Use short sentences and clear timeframes. Avoid copying long blocks of text from previous notes unless clinically necessary. Focus on what changed, what you observed, and what you decided.

3.1 Subjective (S): What the Patient Reports

Document symptoms, timeframe, severity, triggers, and impact on functioning. Use the patient's words for the chief complaint when possible.

Model phrases:

- ▶ "Patient reports ... for ... with impact on ..."
- ▶ "Symptoms worsen with ... and improve with ..."
- ▶ "Sleep averaging ... hours per night."
- ▶ "Denies suicidal or homicidal thoughts."

3.2 Objective (O): MSE and Observations

Document observable findings and the MSE. Keep descriptions objective. Do not label motives or make assumptions.

Model phrases:

- ▶ "Calm and cooperative."
- ▶ "Mood 'anxious'; affect constricted but appropriate."
- ▶ "Thought process logical and goal-directed."
- ▶ "No hallucinations reported; no response to internal stimuli observed."

3.3 Assessment (A): Diagnosis and Clinical Meaning

Model phrases:

- ▶ “Symptoms most consistent with ...”
- ▶ “Differential includes ...”
- ▶ “Rule out ... due to ...”
- ▶ “Risk assessed as low/moderate/elevated based on ...”

3.4 Plan (P): Clear Next Steps

Write the plan as clear actions. Include medication details if applicable, therapy recommendations, education, safety planning, and follow-up timing.

Model phrases:

- ▶ “Continue ...; monitor ...”
- ▶ “Recommend CBT for ...”
- ▶ “Education provided on ...”
- ▶ “Follow up in ... to reassess ...”

4. Objective Language: What to Avoid and What to Write Instead

 **Avoid**

“noncompliant”

 **Use Instead**

“patient reports missing doses due to ...”

 **Avoid**

“manipulative”

 **Use Instead**

“patient requested ... repeatedly; became upset when limits were set.”

 **Avoid**

“drug-seeking”

 **Use Instead**

“patient requested early refill of ...; prescription history reviewed; plan discussed.”

5. Mini Templates (Copy and Paste)

Mini SOAP template:

- S** (CC + HPI + symptom review + key history areas)
- O** (include MSE)
- A** (diagnosis + risk)
- P** (meds/therapy/education/safety/follow-up)

Mini Evaluation template (outline):

- ▶ Chief Complaint
- ▶ HPI
- ▶ Past Psychiatric History
- ▶ Medical History / Medications / Allergies
- ▶ Substance Use
- ▶ Family History
- ▶ Social History
- ▶ Mental Status Exam
- ▶ Assessment (diagnosis + differentials + risk)
- ▶ Plan (treatment + follow-up)

5. Mini Templates (Copy and Paste)

S

Patient reports increased anxiety for 3 weeks with restlessness and poor sleep. Denies panic attacks. Denies SI/HI.

O

Calm and cooperative. Speech normal rate and tone. Mood “anxious.” Affect constricted but appropriate. Thought process logical and goal-directed. No hallucinations noted. Alert and oriented ×4.

A

Symptoms most consistent with Generalized Anxiety Disorder. Risk low. Rule out medical contributors if symptoms persist.

P

Recommend CBT, coping skills practice, reduce caffeine, and follow up in 3–4 weeks. Review return precautions.

Chapter Summary

In this chapter, you learned how to write clear psychiatric documentation using SOAP structure and professional, objective language. You reviewed what belongs in each SOAP section, how to avoid common documentation pitfalls, and how to use mini templates to improve efficiency. Strong documentation supports safety, continuity, and high-quality care.

Practice Exercises

1. Write a mini **SOAP** note for a fictional patient with depressed mood for 2 months and low energy.
2. Rewrite three vague phrases into clear documentation (example: “doing better,” “not cooperative,” “no problems”).
3. Create your own personal list of five documentation phrases you want to use consistently.

Clinical Interviewing: Scripts, Strategies & Tips

Chapter Overview

Clinical interviewing is a skill you build with repetition. In this chapter, you will learn practical scripts and strategies to open an interview, ask sensitive questions, respond with empathy, redirect when needed, and keep the assessment structured. The goal is to help you communicate clearly while maintaining a calm, supportive tone.

What You Will Learn

- ▶ How to open and close a psychiatric interview smoothly
- ▶ Empathy statements that build trust without overpromising
- ▶ How to ask sensitive questions (trauma, substance use, SI/HI)
- ▶ How to redirect and stay on track without sounding harsh
- ▶ How to manage difficult moments (silence, anger, tearfulness)
- ▶ Practical interviewing tips you can use immediately

1. Opening the Interview (Simple, Professional)

Strong openings reduce anxiety and set expectations. Use clear, respectful language and explain what will happen.



Opening Script

Thank you for coming in today. I'd like to understand what you've been experiencing. I will ask questions about mood, sleep, stress, and safety. You can take your time, and you can ask questions at any point.

2. Setting an Agenda (Keeps the Visit Structured)

An agenda helps you stay organized and helps the patient know what to expect. It also reduces interruptions later in the visit.



Agenda Phrases

First, I want to hear what brought you in. Then I'll ask some questions to understand your symptoms. At the end, we will discuss next steps. What is the main concern you want to make sure we address today?

3. Empathy Statements (Simple and Effective)

Empathy statements help patients feel understood. The goal is to validate feelings without validating harmful behaviors.

Examples:

- ▶ “That sounds very stressful.”
- ▶ “I’m glad you told me.”
- ▶ “It makes sense that you feel overwhelmed.”
- ▶ “You’ve been carrying a lot.”

4. Asking Sensitive Questions (Trauma, Substance Use, SI/HI)

Ask sensitive questions with a calm tone and a clear reason. Patients respond better when they understand why you are asking.



Trauma

Some people have been through difficult experiences that still affect them. Has anything like that happened to you?



Substance Use

I ask everyone about alcohol and other substances because they can affect mood, sleep, and anxiety. What do you use, if anything?



Safety (SI)

When people feel overwhelmed or depressed, they sometimes have thoughts of hurting themselves. Have you had any thoughts like that?



Safety (HI)

“Have you had any thoughts about harming someone else?”

5. Redirection Skills (Stay on Track Without Sounding Harsh)

Some patients provide long stories or shift topics frequently. Redirection helps you gather the needed information while maintaining rapport.

Respectful redirection phrases:

- ▶ “That’s important, and I want to come back to it. First, I want to understand your sleep.”
- ▶ “Let me pause you for a moment so I can make sure I understand.”
- ▶ “I hear you. To help me be accurate, I need to ask a few focused questions.”

6. Managing Silence, Tears, and Strong Emotions

Silence can be productive. Give the patient a moment to think. If the patient cries, stay calm and supportive. If emotions escalate, focus on safety and grounding.

Helpful phrases:

- ▶ Take your time.”
- ▶ “It’s okay to pause.”
- ▶ “I can see this is painful. We can slow down.”
- ▶ “Let’s take a breath together for a moment.”

7. When the Patient Is Angry or Defensive

Anger is often a sign of fear, frustration, or past negative experiences with care. Stay calm, set respectful boundaries, and return to the purpose of the visit.

De-escalation phrases:

- ▶ “I can see you’re frustrated. I want to understand what feels upsetting.”
- ▶ “I’m not here to judge you. I want to help.”
- ▶ “Let’s slow down so we can work through this safely.”

8. Closing the Interview (Clear Summary and Next Steps)

A good closing includes a brief summary, the plan, and an opportunity for questions. This improves understanding and engagement.

**Closing Script**

Let me summarize what I heard today. You’ve been experiencing ... for ... and it has affected Based on that, the next steps are What questions do you have?

9. Quick Interviewing Tips (High-Yield)

- ▶ Ask one question at a time.
- ▶ Use short, clear sentences (especially for ESL patients).
- ▶ Reflect back what you heard to confirm accuracy.
- ▶ Document key quotes only when clinically meaningful.
- ▶ Keep a calm tone; your demeanor affects patient regulation.
- ▶ If safety concerns arise, pause the interview and prioritize risk assessment.

Chapter Summary

In this chapter, you learned practical interviewing scripts and strategies to build rapport, ask sensitive questions, redirect respectfully, manage difficult moments, and close the visit clearly. Strong interviewing improves both the quality of your assessment and the patient's trust in the care process.

Practice Exercises

1. Practice the opening script out loud until it feels natural.
2. Write three empathy statements you would use with a patient who feels ashamed or embarrassed.
3. Create a short redirection phrase you can use when a patient goes off topic, and practice it out loud.

Case Studies with Step-by-Step Clinical Reasoning

Chapter Overview

This chapter gives you practice cases that follow the same structure you have learned throughout this book: Subjective (S), Objective/MSE (O), Assessment (A), and Plan (P). Each case includes a brief patient story, a model MSE, differential diagnosis reasoning, and a sample plan. Use these cases to practice writing your own notes and to strengthen your clinical thinking.

What You Will Learn

- ▶ How to apply the **SOAP** structure to real-world psychiatric presentations
- ▶ How to form a top-3 differential diagnosis and document rule-outs
- ▶ How to integrate risk assessment into clinical reasoning
- ▶ How to write a clear, patient-centered treatment plan
- ▶ How to translate interview information into strong documentation

How to Use These Case Studies

1. Read the case story once without writing anything.
2. Read it again and write your own S, O/MSE, A, and P on a blank page.
3. Compare your work to the model solution.
4. Identify what you missed and rewrite your note once more.

Case Study #1 — Depression Presentation

Patient Profile

Adult patient presenting for mood concerns Symptoms present for several weeks No current safety crisis reported

S

S (Subjective)

Patient reports persistent low mood and reduced interest in usual activities for several weeks. Reports low energy and difficulty concentrating. Sleep is disrupted. Appetite is reduced. Denies current suicidal or homicidal thoughts.

O

O (Objective) + MSE

Appearance appropriate; behavior cooperative. Speech soft but coherent. Mood “depressed.” Affect constricted but appropriate. Thought process logical and goal-directed. Thought content denies SI/HI. Perception denies hallucinations. Cognition alert and oriented ×4. Insight fair. Judgment intact.

A

A (Assessment)

Presentation most consistent with Major Depressive Disorder based on low mood, anhedonia, sleep disruption, low energy, and functional impact. Differential includes Generalized Anxiety Disorder (worry may contribute) and Adjustment Disorder (if clearly linked to a stressor). Rule out Bipolar II — no history of decreased need for sleep or elevated mood reported. Risk assessed as low based on denial of SI/HI and intact judgment.

P

P (Plan)

Discuss treatment options using shared decision-making. Recommend psychotherapy (for example, CBT) to address negative thinking and low motivation. Provide education on sleep routine and activity scheduling. Review return precautions and safety resources. Arrange follow-up to reassess symptoms, functioning, and safety.

Practice Exercises

1. Write a one-paragraph clinical summary for this case.
2. List a top-3 differential diagnosis and one supporting reason for each.
3. Write a brief plan that includes therapy, education, and follow-up.

Case Study #2 — Generalized Anxiety Presentation

Patient Profile

Adult patient presenting for chronic worry Sleep disruption and muscle tension No psychosis reported.

S

S (Subjective)

Patient reports excessive worry most days for several months. Reports restlessness, muscle tension, and difficulty sleeping. Worry feels hard to control and affects concentration. Denies panic episodes. Denies SI/HI.

O

O (Objective) + MSE

Appearance appropriate; behavior cooperative; mild fidgeting. Speech normal rate and tone. Mood “anxious.” Affect constricted but appropriate. Thought process logical; ruminative. Thought content denies SI/HI. Perception denies hallucinations. Cognition alert and oriented ×4. Insight good. Judgment intact.

A

A (Assessment)

Presentation most consistent with Generalized Anxiety Disorder given persistent worry with physical symptoms and sleep disruption. Differential includes Panic Disorder (no discrete attacks reported) and Major Depressive Disorder (monitor for low mood and anhedonia). Risk assessed as low due to denial of SI/HI and intact judgment.

P

P (Plan)

Recommend psychotherapy (for example, CBT) focused on worry management and avoidance reduction. Provide education on relaxation practice, reducing caffeine, and sleep hygiene. Discuss monitoring of symptom severity and functioning. Arrange follow-up to reassess progress and safety.

Practice Exercises

1. Write 5 symptom-focused questions you would ask to clarify anxiety severity.
2. Draft a brief Objective section emphasizing observable findings.
3. Write a Plan that includes education and a specific follow-up timeframe.

Case Study #3 — Possible Mania/Hypomania Presentation

Patient Profile

Adult patient presenting with mood swings Periods of decreased sleep reported Impulsivity concerns

S

S (Subjective)

Patient reports episodes of feeling unusually energetic with decreased need for sleep. During these periods, patient talks more than usual and feels more confident. Patient reports increased impulsive decisions. Current mood is “stressed.” Denies SI/HI.

O

O (Objective) + MSE

Appearance appropriate; behavior cooperative but mildly restless. Speech slightly rapid at times. Mood “stressed.” Affect labile. Thought process generally logical but occasionally tangential. Thought content denies SI/HI. Perception denies hallucinations. Cognition alert and oriented ×4. Insight fair. Judgment variable based on history of impulsivity.

A

A (Assessment)

Presentation raises concern for a bipolar-spectrum condition given episodes of decreased need for sleep, increased activity, and impulsivity. Differential includes Anxiety Disorder (can mimic restlessness) and Substance-Induced Mood Symptoms (timing to clarify). Rule out psychosis — no hallucinations or fixed delusions reported. Risk assessed as low at this time due to denial of SI/HI, but monitor judgment and impulsivity.

P**P (Plan)**

Clarify timeline, duration, and impairment level of elevated mood episodes. Recommend mood monitoring and structured routine. Discuss therapy focused on coping and impulse management. Provide education on sleep stability and avoiding triggers. Arrange follow-up to reassess mood pattern, functioning, and safety.

Practice Exercises

1. List key follow-up questions to confirm or rule out bipolar-spectrum symptoms.
2. Write an Assessment paragraph including differential and rule-outs.
3. Identify two safety considerations related to impulsivity and sleep loss.

Case Study #4 — Psychosis Concerns Presentation

Patient Profile

Adult patient reporting paranoia and hearing voices Impaired functioning reported Safety and judgment concerns

S

S (Subjective)

Patient reports feeling watched and reports hearing voices at times. Symptoms are affecting daily functioning. Patient feels fearful in public settings. Denies current SI/HI but reports high distress.

O

O (Objective) + MSE

Appearance disheveled. Behavior guarded; intermittently scanning room. Speech normal volume with delayed responses. Mood “fine.” Affect blunted. Thought process tangential at times. Thought content includes paranoid ideation. Perception: reports auditory hallucinations. Cognition alert and oriented ×3. Insight poor. Judgment impaired.

A

A (Assessment)

Presentation suggests a psychotic disorder given hallucinations, paranoia, impaired insight, and functional decline. Differential includes substance-induced psychosis (substance history and timing to clarify) and mood disorder with psychotic features (assess mood episodes). Risk assessed as elevated due to psychosis and impaired judgment; safety planning and close follow-up required.

P

P (Plan)

Prioritize safety and consider higher level of care per protocol if risk escalates. Clarify substance use and medical contributors. Coordinate referrals as appropriate. Provide education to patient/supports when possible. Establish close follow-up and document return precautions clearly.

Practice Exercises

1. List 5 questions to assess risk related to hallucinations and paranoia.
2. Write a brief safety-focused Plan for this presentation.
3. Identify two rule-outs you would consider and what information you need to clarify them.

Chapter Summary

In this chapter, you applied the full psychiatric assessment process to common clinical presentations using the SOAP structure. You practiced translating patient reports and observed findings into a clear assessment with differential reasoning and a practical plan. Repetition with structured case studies is one of the fastest ways to improve both clinical thinking and documentation skills.

Practice Exercises

1. Choose one case and rewrite it as a full **SOAP** note in your own words.
2. For each case, write a one-sentence primary diagnosis justification.
3. Create a personal checklist of “must ask” questions you will use in every new evaluation.

Skill Workbook: Exercises, Practice Templates & Checklists

Chapter Overview

This workbook chapter helps you practice psychiatric assessment skills using structured exercises. You will find templates you can fill in, checklists you can reuse, and short drills that build confidence. Use this chapter to strengthen your interviewing, MSE documentation, differential diagnosis reasoning, and treatment planning.

What You Will Learn

- ▶ Practical templates you can reuse for real assessments
- ▶ Checklists that help you stay organized and avoid missing key information
- ▶ Drills to strengthen HPI writing, MSE documentation, and diagnostic reasoning
- ▶ Practice worksheets for risk assessment and treatment planning

How to Use These Case Studies

1. Choose one worksheet and complete it in 10–20 minutes.
2. Repeat the same worksheet with a new fictional patient to build speed.
3. Save your completed worksheets as your personal study library.

Worksheet 1 — HPI Builder (Fill-In Template)

Use this to write a clear HPI in a structured way.

Chief Complaint (Patient's Words)

.....

Onset: When did symptoms start?

.....

Duration: How long has it lasted?

.....

Severity: Mild / Moderate / Severe (describe)

.....

Course: Better / Worse / Same over time (explain)

.....

Triggers: What makes it worse?

.....

Relief: What helps?

.....

Function: How does this affect work/school/relationships/self-care?

.....

Safety: Any SI/HI?

.....

Worksheet 2 — Symptom Drill

Worksheet 2 — Symptom Drill (Choose One)

Choose one symptom area and write 5–7 questions you would ask.

☐ Depression

☐ Anxiety

☐ Panic

☐ PTSD/Trauma

☐ Psychosis

☐ Mania

☐ Hypomania

☐ Substance Use

1.

2.

3.

4.

5.

6.

7.

Worksheet 3 — MSE Quick Template

Write a brief MSE using short phrases. Keep it objective.

Appearance

.....

Behavior/Psychomotor

.....

Speech

.....

Mood (patient words)

.....

Affect (observed)

.....

Thought Process

.....

Thought Content (SI/HI, delusions)

.....

Perception (hallucinations)

.....

Cognition (orientation/attention/memory)

.....

Insight

.....

Judgment

.....

Worksheet 4 — Differential Diagnosis Builder

Worksheet 4 — Differential Diagnosis Builder

List your top 3 possible diagnoses and briefly justify each one.

1. Most likely diagnosis

Why?

2. Differential

Why?

3. Differential

Why?

Rule-outs (considered but not supported)

Worksheet 5 — Risk Assessment Checklist

Worksheet 5 — Risk Assessment Checklist

Use this when safety is a concern. Document clearly.

☐ SI assessed

☐ HI assessed

☐ Psychosis-related risk
assessed

☐ Substance-related risk
assessed

Ideation

.....

Plan

.....

Intent

.....

Means/Access

.....

History (attempts/violence)

.....

Protective factors

.....

Risk level

☐ Low

☐ Moderate

☐ Elevated (Explain)

.....

Safety plan/return precautions documented?

☐ Yes

☐ No

Worksheet 6 — Treatment Plan Builder (P)

Worksheet 6 — Treatment Plan Builder (P)

Write a plan as clear actions: what, why, and when.

Medication plan (if applicable)

Therapy plan

Education provided

Referrals / Care Coordination

Safety Plan / Precautions

Follow-up Timeframe

What to monitor next visit

Reusable Checklist — New Evaluation Quick Review

Use this checklist at the end of every new evaluation to make sure nothing important was missed.

- ☐ Chief complaint documented in patient's words
- ☐ HPI includes onset, duration, severity, triggers, relief, and functional impact
- ☐ Past psychiatric history reviewed (diagnoses, meds, hospitalizations, therapy)
- ☐ Substance use assessed
- ☐ Social history and supports assessed
- ☐ Trauma history approached respectfully when relevant
- ☐ MSE completed
- ☐ Risk assessment completed (SI/HI; psychosis/substance risk as needed)
- ☐ Primary diagnosis stated with brief justification
- ☐ Differential diagnosis listed
- ☐ Plan documented with clear follow-up and monitoring

Chapter Summary

In this chapter, you practiced psychiatric assessment using structured worksheets and reusable templates. These tools help you write clearer HPIs, complete stronger MSEs, build differentials, document risk, and write practical treatment plans. Repetition using the same templates is one of the fastest ways to improve.

Practice Exercises

1. Complete Worksheet 1 and Worksheet 3 for a fictional patient with insomnia and anxiety.
2. Complete Worksheet 4 (differential) for a fictional patient with low mood and low energy.
3. Practice the Risk Assessment Checklist using a scenario with passive SI and strong protective factors.

Quick Reference Cheat Sheets (High-Yield)

Chapter Overview

This chapter provides quick-reference cheat sheets you can use during study, clinical preparation, and documentation review. These are designed to be simple, clear, and easy to scan. Use them to refresh your memory quickly before or after patient encounters.

What You Will Learn

- ▶ High-yield prompts for Subjective (S) and HPI collection
- ▶ A quick Mental Status Exam (MSE) checklist
- ▶ Risk assessment quick guide (SI/HI essentials)
- ▶ Differential diagnosis reminders
- ▶ Plan (P) checklist for safe next steps

Cheat Sheet 1 — HPI Fast Guide

Use this checklist at the end of every new evaluation to make sure nothing important was missed.

- ▶ Chief complaint (patient's words)
- ▶ Onset: When did it start?
- ▶ Duration: How long has it lasted?
- ▶ Severity: Mild / Moderate / Severe
- ▶ Course: Better / Worse / Same over time
- ▶ Triggers: What makes it worse?
- ▶ Relief: What helps?
- ▶ Function: Work, school, relationships, self-care impact
- ▶ Safety: SI/HI questions

Cheat Sheet 2 — Symptom Quick Prompts

- ▶ **Depression:** mood, interest, sleep, energy, appetite, guilt/hopelessness, SI
- ▶ **Anxiety:** worry, control of worry, restlessness, tension, sleep, avoidance
- ▶ **Panic:** sudden episodes, physical symptoms, fear of recurrence, avoidance
- ▶ **PTSD/Trauma:** nightmares, intrusive memories, avoidance, hypervigilance, startle
- ▶ **Mania/Hypomania:** decreased sleep, increased energy, pressured speech, impulsivity
- ▶ **Psychosis:** hallucinations, paranoia, delusions, thought control concerns
- ▶ **Substance use:** what, how often, last use, problems caused, attempts to cut down

Cheat Sheet 3 — MSE Quick Checklist

- ▶ Appearance (grooming, hygiene, eye contact)
- ▶ Behavior/psychomotor (calm, restless, slowed, agitated)
- ▶ Speech (rate, volume, tone) Mood (patient words)
- ▶ Affect (observed, range, appropriateness)
Thought process (logical, tangential, circumstantial, disorganized)
- ▶ Thought content (SI/HI, delusions, obsessions, paranoia)
Perception (hallucinations)
- ▶ Cognition (orientation, attention, memory)
- ▶ Insight and judgment

Cheat Sheet 4 — Risk Assessment Essentials

- ▶ **If SI/HI is present, assess:**
Ideation (passive vs active; frequency) Plan (specific or vague) Intent (likelihood of acting) Means/access (weapons, meds, other) History (attempts, self-harm, violence) Protective factors (supports, reasons for living)
- ▶ **High-concern indicators:**
Active intent, plan + access, command hallucinations, intoxication/withdrawal, poor judgment

Cheat Sheet 5 — Differential Diagnosis Reminders

- ▶ Start with symptoms + duration + impact on functioning
- ▶ Ask: What diagnoses fit best? What diagnoses could mimic this?
- ▶ List top 2–3 possibilities with one reason for each
- ▶ Document rule-outs (why a diagnosis is less likely)
- ▶ Consider substance and medical contributors when symptoms are new or unclear

Cheat Sheet 6 — Plan (P) Quick Checklist

- ▶ Medications (if appropriate): name, dose, reason, education, monitoring
- ▶ Therapy: type and focus (CBT, trauma-focused, supportive, etc.)
- ▶ Education: sleep, coping skills, lifestyle, return precautions
- ▶ Referrals: therapy, primary care, substance programs, community resources
- ▶ Safety plan: if risk present, document clearly
- ▶ Follow-up: timeframe + what to reassess

Chapter Summary

In this chapter, you received quick-reference cheat sheets to support efficient psychiatric assessment and documentation. Use these tools to stay organized, strengthen recall, and reduce missed details. Cheat sheets work best when used repeatedly until the structure feels automatic.

Practice Exercises

1. Print or save Cheat Sheet 3 (MSE) and use it to practice writing three brief MSEs.
2. Use Cheat Sheet 4 (Risk) to write a short risk documentation paragraph for a fictional patient with passive SI.
3. Choose one cheat sheet and rewrite it in your own words to strengthen retention.

Glossary (Plain-English Clinical Terms)

Chapter Overview

This glossary explains common psychiatric assessment terms in plain English. Use it to improve understanding, support exam preparation, and help you write clearer documentation. The goal is not to memorize every term, but to recognize them and use them correctly when needed.

What You Will Learn

- ▶ If you see a term you do not recognize in a note, look it up here.
- ▶ If you are writing your own notes, choose terms that match what you truly observed.
- ▶ When in doubt, use simple descriptive language.

Glossary Terms

Affect: What you observe about a person's emotional expression (face, voice, and range of emotion).

Anhedonia: Loss of interest or pleasure in activities the person usually enjoys.

Auditory hallucinations: Hearing voices or sounds that other people do not hear.

Blunted affect: Limited emotional expression; less facial or vocal emotion than expected.

Circumstantial: Thoughts include too much detail but eventually return to the point.

Command hallucinations: Hallucinations that tell a person to do something (may raise safety concerns).

Constricted affect: A narrow range of emotional expression; less variation than expected.

Cognition: Thinking abilities, including orientation, attention, memory, and concentration.

Delusion: A fixed belief that is not based in reality and is not changed by evidence.

Disorganized thought process: Thoughts are difficult to follow; connections between ideas may be unclear.

Flat affect: Very little or no emotional expression.

Goal-directed: Thoughts are organized and move toward a clear point.

Homicidal ideation (HI): Thoughts about harming another person.

Insight: How well a person understands their symptoms and need for help.

Judgment: Ability to make safe decisions and respond appropriately to situations.

Labile affect: Emotions change quickly and may not match the situation.

Mania: A period of abnormally elevated or irritable mood with increased energy and other symptoms such as decreased need for sleep and impulsivity.

MSE (Mental Status Exam): A structured way to describe how a person presents during the assessment.

Paranoia: Strong suspicion that others may intend harm or are watching the person.

Passive suicidal ideation: Wishing to be dead or not wake up, without a specific plan to act.

Perception: How a person experiences the world; includes hallucinations or other sensory experiences.

Pressured speech: Speech that is rapid and difficult to interrupt.

Psychomotor agitation: Increased physical activity such as pacing, fidgeting, or restlessness.

Psychomotor slowing: Slowed movements and reduced physical activity.

Ruminative: Repetitive thinking about worries or problems that is hard to stop.

SI (Suicidal ideation): Thoughts about harming oneself or ending one's life.

Tangential: Thoughts shift away from the topic and do not return to the point.

Chapter Summary

In this chapter, you reviewed common psychiatric assessment terms in plain English. Use this glossary to strengthen understanding and improve documentation accuracy. When you are unsure about a term, default to clear, descriptive language.

Practice Exercises

1. Choose five terms from this glossary and write a sentence using each term correctly.
2. Write a brief MSE paragraph using at least three glossary terms (and make sure they match your observations).
3. Replace two clinical terms with simple descriptive language to practice clarity.

Putting It All Together: Full Psychiatric Evaluation Example

Chapter Overview

This chapter shows how all parts of the psychiatric assessment fit together as a complete evaluation. You will review a full evaluation outline and then see a model example that includes Subjective information, Objective/MSE findings, Assessment with differential reasoning, and a patient-centered Plan. Use this chapter as a reference when you want to see the entire workflow in one place.

What You Will Learn

- ▶ A simple outline for a full psychiatric evaluation note
- ▶ How to keep an evaluation organized and easy to read
- ▶ How to connect the **HPI** and **MSE** to diagnosis and plan
- ▶ A complete model example you can copy and adapt

1. Full Psychiatric Evaluation Outline (Quick Template)

Use this checklist at the end of every new evaluation to make sure nothing important was missed.

- ▶ Chief Complaint (patient's words)
- ▶ History of Present Illness (onset, duration, severity, triggers, relief, function, safety)
- ▶ Past Psychiatric History (diagnoses, meds, therapy, hospitalizations, attempts)
- ▶ Medical History / Medications / Allergies (as reported)
- ▶ Substance Use History (as reported)
- ▶ Family History
- ▶ Social History (living situation, work/school, supports, stressors)
- ▶ Trauma History (as appropriate)
- ▶ Mental Status Exam (**MSE**)

- ▶ Assessment (summary, primary diagnosis, differentials, rule-outs, risk)
- ▶ Plan (meds/therapy/education/referrals/safety/follow-up)

2. How to Keep a Full Evaluation Easy to Read

This is a sample format you can adapt. Keep wording simple and clinically accurate. Adjust details to match your actual patient data and clinical setting.



Chief Complaint

I can't sleep and my anxiety is getting worse.

History of Present Illness (HPI)

Patient reports worsening anxiety for several weeks with restlessness, muscle tension, and difficulty falling asleep. Reports worry is hard to control and affects concentration. Reports fatigue during the day. Denies panic attacks. Denies hallucinations. Denies suicidal or homicidal thoughts. Symptoms are affecting daily functioning.

Past Psychiatric History

Reports prior anxiety symptoms in the past but limited formal treatment. No psychiatric hospitalizations reported. No past suicide attempts reported.

Medical History / Medications / Allergies

Medical history reviewed as reported. Current medications and allergies reviewed as reported.

Substance Use History

Substance use assessed as reported. Patient denies substance use that would explain symptoms.

Family History

Family psychiatric history assessed as reported.

Social History

Living situation, work/school functioning, and social supports assessed. Stressors reviewed.

Trauma History

Trauma history approached respectfully; patient shares limited details at this time.

Mental Status Exam (MSE)

Appearance appropriate; behavior cooperative with mild restlessness. Speech normal rate and tone. Mood “anxious.” Affect constricted but appropriate. Thought process logical and goal-directed; ruminative. Thought content denies SI/HI; no delusions. Perception denies hallucinations. Cognition alert and oriented ×4. Insight good. Judgment intact.

Assessment

Presentation most consistent with Generalized Anxiety Disorder given persistent worry with restlessness, muscle tension, and sleep disruption. Differential includes Panic Disorder (no discrete panic episodes reported) and Major Depressive Disorder (monitor for low mood and anhedonia). Rule out medical contributors if symptoms persist. Risk assessed as low based on denial of SI/HI and intact judgment.

Plan

Discuss treatment options using shared decision-making. Recommend psychotherapy (for example, **CBT**) focused on worry management and avoidance reduction. Provide education on relaxation practice, reducing caffeine, and sleep hygiene. Review return precautions. Arrange follow-up in 3–4 weeks to reassess symptoms, functioning, and safety.

Chapter Summary

In this chapter, you reviewed a full psychiatric evaluation structure and a complete model example. A high-quality evaluation is organized, clear, and clinically meaningful. Use the outline and the example as a template to improve efficiency and consistency.

Practice Exercises

1. Use the outline in Section 1 to write your own full evaluation note for a fictional patient.
2. Rewrite the model Assessment section using different wording while keeping the same meaning.
3. Create a personal “end-of-note” checklist to confirm you documented diagnosis, risk, and follow-up clearly.

Final Review & Next Steps: Study Plan + Implementation Guide

Chapter Overview

This final chapter helps you turn the information in this book into consistent skill. You will review the core workflow, use a simple study plan, and learn how to apply the tools in real clinical settings. The goal is to help you move from “knowing the steps” to “doing the steps smoothly and confidently.”

What You Will Learn

- ▶ A full end-to-end review of the psychiatric assessment workflow
- ▶ A simple study plan you can follow (short sessions, high repetition)
- ▶ How to use checklists, templates, and cheat sheets in daily practice
- ▶ A practical “next steps” roadmap for ongoing growth

1. End-to-End Workflow Review (One Page Summary)

Use this workflow as your mental map for every encounter. Over time, it becomes automatic.

Psychiatric assessment workflow:

- ▶ Open the visit, build rapport, set agenda
- ▶ Subjective: CC + HPI + symptom review + key history areas
- ▶ Objective: observations + MSE
- ▶ Risk assessment: SI/HI and related risks when indicated
- ▶ Assessment: summary + primary diagnosis + differential + rule-outs + risk level
- ▶ Plan: treatment + education + referrals + safety + follow-up
- ▶ Document clearly and objectively

2. A Simple Study Plan (Low Time, High Impact)

The fastest way to improve is short, repeated practice. You do not need long study sessions. You need consistent repetition with the same core templates.

Example weekly plan:

- ▶ 2 days/week: write one HPI using Worksheet 1 (10 minutes)
- ▶ 2 days/week: write one MSE using Worksheet 3 (10 minutes)
- ▶ 1 day/week: build a differential using Worksheet 4 (10–15 minutes)
- ▶ 1 day/week: write a full mini SOAP note (15 minutes)
- ▶ 1 day/week: review cheat sheets and glossary (10 minutes)

3. How to Use These Tools in Real Clinical Work

In practice, you will not use every worksheet in every visit. Instead, use the tools as supports when you need structure, speed, or confidence.

Practical use examples:

- ▶ Before a new evaluation: review Cheat Sheet 1 (HPI) and Cheat Sheet 3 (MSE)
- ▶ During an evaluation: keep the MSE checklist nearby to avoid missed sections
- ▶ After the visit: use Worksheet 4 to structure your differential and rule-outs
- ▶ When safety concerns arise: use the Risk Assessment Checklist and document clearly
- ▶ When writing plans: use the Plan checklist to include follow-up and monitoring

4. Building Speed Without Losing Quality

Speed comes from structure. Use the same format repeatedly. Write shorter notes first, then improve detail as needed.

Speed tips:

- ▶ Use templates to reduce decision fatigue
- ▶ Write in short phrases instead of long narratives
- ▶ Document timeframes and functional impact consistently
- ▶ Use objective language (describe what you observe)
- ▶ Use model phrases to avoid starting from scratch

5. Next Steps Roadmap

Use the roadmap below to continue improving after you complete the book.

Next steps:

- ▶ Continue practicing one full **SOAP** note per week using the workbook templates
- ▶ Review one case study weekly and rewrite it in your own words
- ▶ Build a personal “phrase bank” of 15–20 documentation phrases you use consistently
- ▶ Practice interview scripts out loud until they feel natural
- ▶ Use the glossary to strengthen accuracy in terminology

6. Closing Notes

Psychiatric assessment is a skill that improves through repetition. If you follow the structure and practice consistently, your confidence and clinical clarity will grow quickly. Use the tools in this book as scaffolding—over time, you will need them less, because the structure will become your default clinical workflow.

Chapter Summary

In this chapter, you reviewed the full psychiatric assessment workflow and learned how to use the tools in this book as a practical system. You also received a simple study plan and a roadmap for continued improvement. Consistent, short practice sessions are the key to long-term skill growth.

Practice Exercises

1. Choose a fictional patient and complete Worksheet 1 (HPI), Worksheet 3 (MSE), Worksheet 4 (differential), and Worksheet 6 (plan).
2. Write a full mini SOAP note from your worksheets in under 20 minutes.
3. Create your own weekly practice schedule and commit to it for 2 weeks.

Professional, Ethical, and Legal Considerations in Psychiatric Assessment

Chapter Overview

Psychiatric assessment is not only a clinical skill—it is also a professional, ethical, and legal responsibility. This chapter reviews high-yield ethical principles, legal considerations, and professional boundaries that guide safe and effective psychiatric practice across settings.

What You Will Learn

- ▶ Core ethical principles relevant to psychiatric assessment
- ▶ Legal responsibilities related to safety, documentation, and consent
- ▶ Professional boundaries and scope of practice awareness
- ▶ Risk management strategies that protect patients and clinicians

1. Core Ethical Principles

Ethical decision making guides every psychiatric assessment. Key principles include beneficence (acting in the patient's best interest), nonmaleficence (avoiding harm), autonomy (respecting patient choice), and justice (providing fair and equitable care). Assessments should be conducted with respect, cultural awareness, and attention to power dynamics.

2. Informed Consent and Transparency

Patients should understand the purpose of the assessment, the limits of confidentiality, and how information will be used. Informed consent is an ongoing process, not a single statement. When capacity is in question, document clearly and follow institutional policy.

3. Confidentiality and Its Limits

Confidentiality is foundational, but it has limits. Situations involving imminent risk to the patient or others, abuse or neglect, or court orders may require disclosure. These limits should be explained to patients early and documented clearly.

4. Documentation as a Legal Record

Psychiatric documentation is a legal document. Notes should be factual, objective, and free of judgmental language. Document what was assessed, what was decided, and why—especially in cases involving safety, refusal of care, or escalation.

5. Scope of Practice and Professional Boundaries

Clinicians must practice within their training, licensure, and institutional scope. Know when to consult, refer, or escalate care. Maintain clear professional boundaries to preserve therapeutic integrity and patient safety.

6. Risk Management and Clinical Judgment

Risk management does not mean avoiding difficult cases—it means approaching them systematically. Use structured assessments, consult when uncertain, follow protocols, and document your reasoning. When in doubt, prioritize safety and supervision.

Chapter Summary

In this chapter, you reviewed the ethical, legal, and professional foundations of psychiatric assessment. Strong clinical skills must be paired with ethical awareness, legal knowledge, and professional judgment. These principles protect both patients and clinicians and support high quality psychiatric care.

FAQs, Common Mistakes, & Preceptor Red Flags

Chapter Overview

This chapter focuses on the practical issues that frequently show up in training and early practice: common questions, documentation mistakes, and the red flags that preceptors often correct. Use this chapter to improve clarity, reduce rework, and strengthen your clinical confidence.

What You Will Learn

- ▶ High-yield answers to common psychiatric assessment questions
- ▶ The most frequent mistakes that weaken notes and clinical reasoning
- ▶ Preceptor “red flags” that signal missing structure or unsafe assumptions
- ▶ A simple correction method you can apply immediately

1. Frequently Asked Questions (FAQs)

Use this workflow as your mental map for every encounter. Over time, it becomes automatic.

Q

How long should a new evaluation note be?

A

Long enough to be clinically meaningful, but short enough to be readable. Focus on timeframes, functional impact, MSE, risk, diagnosis reasoning, and plan.

Q

What if I do not know the diagnosis yet?

A

Document your clinical summary, list a reasonable differential, explain what information is missing, and state your follow-up plan to clarify.

Q

Do I have to document every symptom question I asked?

A

No. Document key positives/negatives that support your reasoning, especially safety-related findings and symptoms that drive diagnosis.

Q

How do I write a strong Assessment section?

A

Include a brief clinical summary, your primary diagnosis with a short justification, your top differentials with rule-outs, and your risk level.

Q

What should I do if the patient is vague or inconsistent?

A

Document what was reported, clarify timelines, reflect uncertainty honestly, and support your assessment with observable findings and collateral when available.

2. Common Mistakes (and How to Fix Them)

 Mistake

Writing long narrative paragraphs with no structure.

 Fix

Use headings and bullet points. Keep HPI structured (onset, duration, severity, function, safety).

 Mistake

Missing functional impact.

 Fix

Add 1–2 lines: how symptoms affect work, school, relationships, self-care.

 Mistake

Skipping risk language when SI/HI is denied.

 Fix

Document explicitly that SI/HI was assessed and denied; state risk level when clinically relevant.

 Mistake

Using judgmental language (e.g., “noncompliant,” “manipulative”).

 Fix

Use objective descriptions (e.g., “missed doses,” “requested early refills,” “became frustrated”).

 Mistake

Listing diagnoses without reasoning.

 Fix

Add one sentence linking symptoms + duration + impairment to the diagnosis, and a brief differential/rule-out statement.

Chapter Summary

In this chapter, you reviewed common questions, frequent documentation mistakes, and preceptor red flags. Use the correction method and checklist to strengthen structure, clarity, and safety in every evaluation.

Capstone Toolkit: One-Page Workflow, Phrase Bank, & Self-Check

Chapter Overview

This chapter is a capstone toolkit you can use repeatedly. It includes a one-page clinical workflow, a personal phrase bank you can customize, and a self-check system to confirm you covered the essentials in every assessment. Use it as your final “carry-forward” resource after completing the book.

What You Will Learn

- ▶ A one-page psychiatric assessment workflow you can follow in any setting
- ▶ A phrase bank for documentation and patient communication
- ▶ A self-check tool to reduce missed details and strengthen clinical reasoning
- ▶ A quick “preceptor-ready” review routine you can apply before clinical days

1. One-Page Workflow (Print and Keep Nearby)

Use this condensed workflow as your mental checklist for every encounter. The goal is consistent structure, not perfect wording.

Psychiatric assessment workflow (one-page):

- ▶ Open: rapport + agenda + confidentiality limits (brief)
- ▶ Subjective: CC + HPI + symptom review + key history areas
- ▶ Objective: observations + MSE
- ▶ Safety: SI/HI and risk factors/protective factors (when indicated)
- ▶ Assessment: clinical summary + primary diagnosis + top differentials + rule-outs + risk level
- ▶ Plan: treatment + education + referrals + safety plan + follow-up timeframe
- ▶ Document: clear, objective language; rationale for key decisions

2. Personal Phrase Bank (Copy, Edit, Reuse)

Phrase banks reduce documentation time and improve consistency. Use the examples below as a starting point and tailor them to your style.



2.1 Patient Communication Phrases

- *Thank you for telling me. I want to understand this clearly.*
- *I'm going to ask a few focused questions so I can be accurate.*
- *Let me summarize what I heard to make sure I got it right.*
- *We will review next steps together before we finish today.*



2.2 Documentation Phrases (Objective and Clear)

- *Patient reports ... for ... with impact on ...*
- *Denies suicidal or homicidal thoughts.*
- *Thought process logical and goal-directed.*
- *Risk assessed as low/moderate/elevated based on ...*
- *Differential includes ...; rule out ... due to ...*



2.3 Redirection and Structure Phrases

- *That's important, and I want to return to it. First, I need to ask about ...*
- *Let me pause you so I can make sure I understand.*
- *To help me be accurate, I'm going to ask a few short questions.*

3. Self-Check: End-of-Visit Quality Review

Use this quick self-check at the end of every evaluation or follow-up note. It improves safety and reduces missed details.

- ▶ CC documented in patient's words
- ▶ HPI includes onset, duration, severity, triggers/relief, and functional impact
- ▶ Key symptom domains assessed (as relevant)
- ▶ Substance use assessed when clinically relevant
- ▶ MSE documented with objective findings
- ▶ SI/HI assessed when indicated; risk level stated
- ▶ Primary diagnosis stated with brief justification
- ▶ Differential diagnoses considered; key rule-outs noted
- ▶ Plan includes education + follow-up timeframe
- ▶ Documentation is objective (no judgmental language)

4. Preceptor-Ready Routine (5–10 Minutes)

Use this quick routine before clinical days or presentations to improve confidence and flow.

5–10 minute routine:

- ▶ Review the One-Page Workflow (Section 1).
- ▶ Choose 3 phrases from the Phrase Bank you want to use that day (Section 2).
- ▶ Skim the MSE checklist and risk essentials from your cheat sheets.
- ▶ After your first patient, complete the Self-Check once to confirm structure.

Worksheet: Build Your Own Phrase Bank

Worksheet: Build Your Own Phrase Bank

Fill this in with your preferred wording. Keep the final version in a note app or printed binder.

Opening script

Empathy statement #1

Empathy statement #2

Redirection phrase

Risk documentation phrase

Differential phrase

Closing script

Chapter Summary

In this chapter, you created a practical capstone toolkit: a one-page workflow, a customizable phrase bank, and a fast self-check. These tools are designed to make your assessments more consistent, your documentation clearer, and your clinical days less stressful.

The Ultimate Clinical Companion

PSYCHIATRIC ASSESSMENT MASTERY

Psychiatric Assessment Mastery bridges the gap between textbook theory and real-world clinical practice. Designed specifically for PMHNP students, registered nurses, and new practitioners, this practical workbook provides the scripts, templates, and decision-making frameworks needed to assess patients safely, efficiently, and with confidence. Rather than memorizing disconnected facts, you'll learn how to think clinically—from the first interview question to a well-reasoned diagnosis and clear documentation.

With this book, you will be able to:

Perform a structured, confident Mental Status Examination (MSE)

Write clear, exam-safe SOAP notes and treatment plans

Navigate DSM-5-TR diagnostic families with clinical reasoning

Identify risk factors and red flags that guide safe decision-making

Translate assessment findings into real-world clinical documentation

“Finally, a resource that explains how to think, not just what to memorize. The interview scripts and differential diagnosis frameworks were invaluable during clinical rotations.”



Patrick Brook, MD

Clinical Preceptor, PMHNP-BC



About The Author

Tonia Ojomo, MSN,BSN, RN.

Tonia Ojomo is an experienced psychiatric mental health nurse dedicated to clinical excellence. She created the Psychiatric Assessment Mastery system to help students and clinicians navigate the complexities of mental health evaluation with clarity, safety, and confidence.

For educational purposes only.

Not a substitute for clinical supervision or independent clinical judgment.